CONSULT FOR AFRICA

The Growth Pack

Business Development and Growth

Prepared for Dorathy Michael, Director of Business Development and Growth
Consult for Africa internal. Not for circulation outside the firm.
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A letter from Debo

Dorathy,

This is the longest document anybody at this firm has been handed, and I want to explain why you are the one getting it.

Consult for Africa has a demand problem that does not look like a demand problem. We are carrying more engagements than we have people, almost every one of them larger now than when it began, and the book has grown every month this year. From the inside that reads as success. What it actually is, is a firm living entirely off one person's relationships, which is me, and that is not a growth engine, it is a bottleneck with good manners. Every engagement in Part Three arrived because somebody I worked with at Cedarcrest or Evercare or through Doctoora picked up the phone. None of it arrived because the firm went and found it.

Alongside the consulting we have built products, and the reason matters. The same three things break in every African health institution we are called into, which are leadership, workforce and the economics of a practice, and I got tired of recommending fixes that nobody in this market could actually buy. So we built them. Maarova for leadership, CadreHealth for workforce, Mezo for the practice. They work. They are built, they are deployed, they hold real data and real users, and between them they have produced almost no revenue, because building a product and selling one are different jobs and we have only ever done the first.

That is the job. You own the growth of the firm's own revenue and you own the growth of the products, and those are not two jobs loosely related, they are the same job seen from two ends, because the products are what let us sell to a hospital that cannot afford a consulting engagement and the consulting is what gets the products into a building where somebody will use them.

A few things I want to say plainly.

I am not asking you to run a sales team, because there is not one, and the first thing you will probably tell me is that there needs to be. What I am asking for is that the pipeline stops being a function of my diary. Read Part Four before you read anything else, because it is the shortest part of this document and the most uncomfortable: our pipeline on the platform holds one lead, which has been sitting untouched since the eighteenth of September, and four proposals that have been in draft since March. That is not a tooling problem. It is what happens when nobody owns the pipe.

On the products, you are going to find that the problem is almost never the product. It is that nobody was told. Part Five sets out the measurement that makes this unarguable: on Mezo, conversion is about sixty eight per cent among the doctors who were actually told they had a place, and zero out of nine hundred and seventy among those who were not. Every product in the firm has a version of that number, and the lever is the same lever each time.

I will give you the relationships. Ask me how I know somebody before you write to them, every single time, because I usually do know them and a correct but cold letter to a friend of fifteen years costs more than a late one. That is not a formality and it is the one rule I will be irritable about.

The last thing is confidentiality, which I am strict about. Everything here is real, including the client names, the findings and the shape of the deals, because you cannot sell what you do not understand. None of it leaves the firm. We describe our work by method and lesson, never by client, and in a market this small a combination of harmless details identifies somebody.

On timing. You are full time from January, so this pack is deliberately arriving three months early, and that is the best thing about it. Nobody gets to read their firm properly before they start. You do. Read Parts Four and Five first, then Three, then the rest, and do it across the next few weeks rather than the next few days.

What I do not want is for the January start to read as three quiet months. Two things are worth doing now. Tell me where you think I am wrong, because a view formed before you are inside the machine is worth more than one formed after. And keep the Aman engagement moving, because that is already yours and it is the only part of this book you have actually run.

The things in this pack with dates on them are mine until January, not yours. Part Eight says which ones and who is holding each, because a handover list that assumes you are already here would have you worrying in October about work you cannot touch until the new year.

The natural first step is for you and me to sit down and settle three things: what we do about the pipeline between now and January, whether the products get a campaign or a parking space, and your own terms, which Part Two is honest about being unsettled.

Warm regards,

Dr Debo Odulana

Founding Partner, Consult for Africa

How to use this pack

Part	What it covers	When you need it
1	The firm: what we sell, how we are bought, how we price	Day one. Read once, properly.
2	Your role, what you own, and what is unsettled about it	Day one, alongside Part One.
3	The clients: every live engagement and the growth in each	Week one. This is the revenue that exists.
4	The pipeline, and the honest state of it	**First.** It is four pages and it is the job.
5	The products, which are yours: Maarova, CadreHealth, Mezo and the rest	**Second.** The longest part, because it is the least understood.
6	The growth arithmetic: which lever is worth most	Week one, then again monthly.
7	How we sell, how we price, and the rules on writing	Before you send anything to anybody.
8	What is held by whom until January, and your first ninety days from there	Now, for the handover list. Again in January.
A to E	Directory, glossary, the channels, the checklists, where things live	Whenever you are unsure.

Two conventions, and one deliberate absence.

Where a fact needs confirming rather than knowing, it says so in the text, and those are flags rather than gaps somebody forgot to fill. Where a number is given, it was read off the live platform on 2 October 2026, which is three months before you start, so by January every figure in Part Five in particular will have moved. Treat them as the shape of the problem rather than as its measurements, and re read them off the platform when you arrive.

The absence is money. **This document carries no fees, no rates and no client balances**, because fee structures are the most sensitive commercial information the firm holds and a document travels in a way a login does not. The figures are not being withheld from you, they are being kept out of a mailbox: they sit on the platform behind a login, and section 1.6 tells you where and when. What this pack carries instead is the pricing logic, which is the part that survives a change of number and the part you can usefully argue with

before January.

Part One: The firm, and what we actually sell

1.1 What Consult for Africa is

Consult for Africa is a healthcare management consulting firm founded in 2024, working from Lagos and Abuja across Nigeria and into the wider continent. It is written **Consult for Africa** in body copy and **CFA** in short form, and you will also see **C4A** in older files and file names. Both are us, and CFA is preferred in anything new.

The legal billing entity is **Consult for Africa Management Services Limited**, and that is the name on invoices and contracts. It does not appear in marketing copy.

The one line version, which is the one to use in a room:

> Consult for Africa helps hospitals, health systems, investors and governments turn ambition into delivery. Built by clinicians and operators who know the system from the inside, we do not hand you a report and walk away. We build, staff, and stand behind the outcome.

The tagline on the brand guide is **Healthcare performance, restored.**

1.2 The argument the firm rests on

Across the continent the gap between a struggling hospital and a thriving one is rarely the building or the equipment. It is execution, leadership and people. That is not a slogan reverse engineered for a website, it is what every engagement has found independently, which is why a paediatric centre that looked like it had a stock control problem turned out to have an incentives problem, and a surgeon who believed his growth was limited by his clinic's postcode turned out to be limited by a referral pipeline he had never built.

Debo's blunter version is worth remembering, because it is also a sales argument: the presenting cause is always a thing, and the reason is never the thing. A client who tells you their problem is the equipment is telling you what they have already tried to fix.

1.3 The seven published service lines

These have their own pages on consultforafrica.com, and you should read them once so you recognise the language a prospect will have read before they call.

Service line	What it actually means	Typical buyer
Clinical Governance	Safety culture, standards of work, clinical audit, incident systems, accreditation readiness	Medical director, board quality committee
Strategy and Growth	Market entry, growth strategy, service line design, pricing and tariff, feasibility, financial modelling	Owner, group chief executive, board
Fractional Leadership	Senior operators placed part time into a client: interim chief operating officer, medical director, commercial lead	Owner who cannot yet afford or find a full time hire
Digital Health	System selection, records and information system advisory, digitisation programmes, product strategy	Chief executive, chief information officer, hospital group
Health Systems Strengthening	Programme design and delivery for payers, ministries, federations and donors	Government, health insurer, development finance

Service line	What it actually means	Typical buyer
Diaspora Expertise	Access to senior Nigerian clinicians abroad for advisory, teaching, case review and episodic clinical work	Hospital needing scarce specialist capability
Healthcare Marketing Agency	Positioning, brand, digital and patient acquisition for healthcare providers	Clinic, hospital, specialist practice

1.4 The five things clients actually sign

The published seven are the menu. These five are what appears on contracts, and knowing the difference tells you what you are really selling in any given conversation.

What it is	What it looks like week to week	Live examples
The organisational audit	A structured diagnostic across ten domains, a site visit, surveys of staff, patients and referrers, then a report with findings and decisions for the board	Osteon, Arabella, Haven phase two
Turnaround and growth mandate	A target, a set of levers, and delivery against it, usually with us inside the business	Haven phase three, House of Refuge
Board governance	Standing up a board, its committees, delegation of authority, director training	Havana Specialist Hospital
Venture structuring	Holding companies, joint ventures, cap tables, shareholder agreements, alongside the client's counsel	Kemi Balogun group, the aesthetics vehicle
Build and operate	We design the facility or the venture, set it up, then manage it under a management agreement	Medbury division, Lyfe Place Abuja, ile

The audit is the commercial engine of the firm and the single most important thing for you to understand, because it is small, it is buyable, and it reliably finds something larger than its own brief. Arabella is the clearest live example: the audit that began today is explicitly the qualification round for a management contract proposed from month three, worth a multiple of the audit over a year. Haven went audit, then culture build, then a growth mandate. Osteon went positioning, then audit, and the audit is now the gate to everything else.

So when you are selling, the audit is almost always the right first ask, not because it is cheap but because it is the only product we have that a sceptical owner can say yes to without betting the business.

1.5 How demand actually arrives, and what that means for you

Almost entirely by referral and through Debo's own network. That is the single most important fact about this firm's commercial position, and it cuts two ways.

It is why the win rate is high and the sales cycle is short, because a prospect who arrives through a mutual friend is already most of the way sold. It is also why the pipeline is thin, lumpy and unforecastable, because nobody can schedule a referral.

The progression that recurs has five stages and it is worth knowing by name, because each stage has a different deliverable and a different conversation.

The options note comes first, before a prospect has engaged us at all. It is a short note laying out the strategic options, giving capital and revenue as ranges rather than point estimates, stating what has to be

true and what kills each option, and being explicit about what we cannot yet know. It is deliberately not a feasibility study, because a confident twenty six page plan answers every question and leaves nobody a reason to call. This is your most important document and you will write a lot of them.

The diagnostic is where we go in and find out what is actually there, which is where our methods live and where almost all our best proof comes from.

The strategy or the plan is built on the diagnostic and presented as decisions with options rather than recommendations, so the board is choosing rather than being told.

Delivery is execution with a named team and a milestone schedule.

Operate is where we run it under a management agreement with an operator fee, and it is where the annuity is.

Not every engagement goes the whole way and several have. The commercial implication is that our case studies are rarely "we advised and left", and that is our single biggest differentiator against both the international firms and the local boutiques. It is also why the operate stage matters more than its share of today's revenue suggests: a management agreement is recurring, it compounds, and it is very hard for a client to unwind.

1.6 How we price, and where the rate cards live

The numbers are not in this document. The logic is, and the logic is what you will actually argue with.

Resourced days. Delivery work is priced as named people for a number of days at published grades, plus a programme rate on top, which is currently ten per cent. This is the base for audits, strategy and setup work. It matters because it means a discount conversation is a conversation about scope or seniority, not about a mysterious number.

Standard, then one visible concession, then net. Our proposals show the full standard fee, a single named concession line, and the net. Full transparency about what has been given up. This is deliberate, it works, and it is why we never show a pre discounted price as though it were the price.

Anchor, do not open at the floor. We publish an opening position and hold concessions back for the negotiation. We never volunteer a cap, never benchmark ourselves at the bottom of a comparator range in a client document, and never defend a price before anybody has questioned it. Several files have a private negotiation guide in the repository carrying the floor and the concession ladder, and those are never sent to a client under any circumstances.

Build and operate has five separate bases, because it is five different activities: delivery, capital efficiency, establishment, management and equity. One finding from that work is worth carrying into any venture conversation: a percentage of capital development fee is the wrong instrument whenever the client wants lean capital, because it pays us for spending their money. We replaced it with a fixed delivery fee plus a share of the capital saved.

Performance and success fees are where Debo is taking the firm. House of Refuge runs a fundraising mandate charged as a share of funds raised. Dennis Ashley is priced as a share of collections. Arabella's forward path is a share of monthly revenue with a floor in the early months. The Medbury division carries a performance share payable only when the board's revenue goal and its margin goal are both met. If you are shaping a new deal, that is the direction of travel and Debo will back you for proposing it.

Where the figures are. On the platform, at **/finance/rate-card**, which exists for this role. It carries the published day rates by grade, the programme rate, the recruitment position and the one it replaced, the leadership assessment and coaching menu with the coach economics, the member side subscription, the five bases of a build and operate mandate shown as anchor against walk away, the negotiation doctrine, the

terms never to concede, and the path to every private concession ladder the firm holds. It also carries the two calibration points that tell you where the floor really is, which matter more than the card itself.

It opens for you in January, with the rest of your access, and there is no need to chase it before then. The page is gated to the senior roles rather than to Finance, because the office reads invoice status in order to chase and sees no rates at all, and that grant goes with the start rather than ahead of it. Nothing you can usefully do between now and January requires a number off it.

Two things are not on it, on purpose. Client balances and agreed fees, because those live on the engagement record and under Invoices, which is the only place they are ever current, so read a client number there on the day you quote it. And agency pricing, which does not exist yet, covered in Part Five.

One rule that applies to you more than to anybody else in the firm: **never put a price in marketing copy and never defend a price nobody has challenged.** If a piece of content provokes a fee conversation, that is the correct outcome, and the conversation happens on a call.

1.7 The people

Eighty two people hold platform accounts, of which sixty nine are consultants on the bench rather than staff. The names you will deal with weekly are short.

Person	What they hold, and what you need from them
Dr Debo Odulana	Founding Partner. Former chief executive of Cedarcrest Hospitals Abuja, former chief innovation and strategy officer at Evercare Hospital Lekki, founder of Doctoora which he built to twenty one states and sold to Evercare, President of the Doctors Foundation for Care. He is the relationship source for almost every name in Parts Three and Four.
Tito Ipinmoye	Director of Product and Strategy. Runs day to day execution on audit engagements, so she is who you talk to about whether we can actually deliver what you are about to sell.
Toluwanimi Olayeni, called Nimi	Marketing and communications lead, and she runs the healthcare marketing agency as a service line. Your closest working relationship. The published proof you need in order to sell sits with her.
Ejiro Alechenu	Marketing, joined September 2026, works to Nimi.
Oludayo Olugbamigbe	HR Practice Lead, joining from the Nigerian Exchange Group. A psychologist before an HR practitioner, which is why assessment and culture measurement sit inside the practice rather than being bought in. She owns delivery on recruitment and workforce mandates that you sell.
Dr Olufunbi Okeleye	Engagement Manager.
Ezinne Orji	Executive Assistant to the Founding Partner, Abuja, started 2 October 2026. She owns the Thursday pipeline sweep and the commitment register, so she is the person who will chase what you sell. Abigail Oladejo reports to her.
Kola Momodu	On site Project Owner for Arabella, five days a week in Asokoro. Independent contractor, lead auditor in information security, nearly nine years at Cedarcrest Abuja.

There are eight directors on the platform, including Matthew Aloba, Dr Adediwura Okeleye, Barr. Osahon Omoruyi, Bukunola Oyedele, Irene Nwaukwa, Olufemi Shonowo, Patience Ugbaja and Tito Ipinmoye. Several hold practice or faculty roles rather than full time posts. Confirming which of them is actually available to you for a client conversation is worth doing early in January, because a director on a pitch changes how it lands.

1.8 The wider stable

Beyond the firm and the three products, CFA holds or has built a number of ventures. Most are not yet things you may talk about publicly, and the rule of thumb is that if it does not have a live public page you can point at, it is not ready to be sold.

Venture	What it is	Commercial status
Cooked Indoors	Medical nutrition and clinically informed catering, Lagos, live at cookedindoors.com	Publishable. Named in our own advertising already.
Covally	Healthcare cooperative for hospital staff, founded by Debo	Internal until launch
Connexxum	Business to business healthcare procurement, pooled buying, escrow	Internal until beta
Cureva	Medical tourism marketplace with a patient advisor	Internal until pilot
HospitlOS	Hospital operating system used inside managed facilities, with a staff application carrying payroll, recognition, anonymous pulse surveys and onboarding	Internal. Never named in client copy. Built and typechecking, not yet deployed against a database, so a demo needs an environment stood up first.
OrthoSurplus	Orthopaedic implant supply for West Africa, orthosurplus.com	Thin public presence. Clear anything with Debo.
ile	Residential eldercare and home care venture, in build with a client	Own brand, deliberately carries no CFA identity. Do not associate them publicly.
Maarova Vet	Executive integrity vetting, parked pending research	Do not mention at all.
Doctors Foundation for Care	Debo's diaspora physician network, more than five hundred doctors	The convening vehicle, and a real distribution asset for the products

Part Two: Your role, and what is unsettled about it

2.1 What the role holds

The title in the firm's own documents is **Director of Business Development and Growth**, and the handbook describes you as owning the business development practice and the payer and corporate relationships. In practice the role has four pieces and they are not equally built.

The firm's pipeline. Leads, discovery calls, proposals, qualification, and the follow through on all of it. This exists and is in poor condition, which Part Four covers.

The products' growth. Maarova, CadreHealth, Mezo, and the channels that were built to distribute them and never used. This is the largest piece of genuinely new ground and it is where the most value is sitting unclaimed.

Payer and corporate relationships. Health insurers, corporate accounts, employers. You are already the practice lead on the Aman engagement, which is a payer network build, so this is the piece you have actually done here.

The commercial shape of new deals. Not setting price, which is Debo's, but proposing structure, because a growth role that cannot shape a deal is a lead generation function with a better title.

2.2 What the platform says about you, and what changes in January

You have held a platform account since the fifteenth of May 2026 under your own email address. Your profile records you in Abuja, ten years of experience, tier experienced, primary specialty business development, specialty category strategy, with expertise logged in health insurance, revenue cycle, marketing, content and training. Your bio describes healthcare business development, health insurer partnerships and revenue cycle management, with a track record on corporate partnerships and commercial transformation across hospital networks. That is all correct and it is a good record.

Three things about it are out of date, and none of them is urgent, which is why they are listed here rather than in the January checklist.

Your platform role is CONSULTANT and your recorded title is Business Development Manager. You are Director of Business Development and Growth, which is how the marketing handbook and the Aman strategy both name you, so the record is simply behind. It is worth knowing rather than fixing today, because role is what gates access here: the consultant role reaches the consultant surfaces and not the commercial ones, so the pipeline and the rate card are not open to you yet. That changes when you start, as one act rather than as a series of requests, and it is deliberately tied to the start date rather than run ahead of it.

What it means in the meantime is practical. If a page tells you that you do not have access, it is the role rather than a fault, so do not spend time on it. The Aman engagement and your consultant surfaces work as they always have.

No rates are set on your profile and no CV is on file. Both fields are empty, which matters once you are resourced onto a client engagement as a named person on a resourced day schedule, because the schedule prices off the grade. Worth sending Debo a CV before January so it is not being chased in your first week.

2.3 Who owns what, so you are not negotiating boundaries in month three

This is the part most likely to cause friction, so it is written down.

You and Nimi. Marketing's job in a referral led business is not to generate cold leads, it is to make the firm findable, give a referrer something to send, shorten the time from first conversation to signature by having the proof already published, and build the standing that makes an owner take the call. So Nimi builds the proof and you use it. The highest value thing marketing can do for you is clear named references, and right now exactly one is cleared, which is that the firm is the management partner for a twenty one bed drug and addiction rehabilitation centre in Lekki. There is a library of ten anonymised case studies written up and not one has been published. Getting two or three more names cleared is a joint piece of work and the moment to ask a client is when an engagement is going well, not months later.

You and Oludayo. You sell workforce and recruitment mandates, she delivers them. The pricing position on recruitment is hers to hold and Debo's to set, and it moved in September, so read section 5.5 before you quote anybody.

You and Tito. She runs delivery on audits. Before you promise a timeline on a diagnostic, it goes past her, because the firm's capacity is the real constraint on how much you can sell and it is not visible from the commercial side.

You and Ezinne. She runs the Thursday pipeline sweep, which chases every lead untouched for fourteen days and every proposal older than a week. She owns the hygiene and the chasing. You own the deal. That division only works if the pipeline record is current, which means the discipline of logging a conversation is not administrative tidiness, it is what makes somebody else able to chase for you.

You and Debo. He owns price and he owns the relationships. You own structure, qualification and follow through. Where the two meet is the concession ladder, and the rule there is that you never volunteer the

next concession.

2.4 How Debo works

He is fast, he wants things done rather than discussed, and he does not want trailing summaries. A short, specific update beats a complete one. If a thing is decided, say what was decided; if it is stuck, say what you need from him in one line.

He reads on his phone between meetings, often late. Anything that needs a decision should be answerable with a yes, a no or a number, and if it genuinely cannot be, it belongs on the decisions register with the options and your recommendation written out rather than in a message.

He often knows the person you are writing to, frequently from Cedarcrest, Evercare or Doctoora. Always ask how he knows somebody before you draft anything that goes to them. This is the rule he will be most irritable about, because a correct but cold letter to a friend of fifteen years costs more than a late one, and in this market there is no recovering from it.

2.5 What is unsettled, and should be settled early

Your own terms are not recorded anywhere the platform can see, which is the same gap as the missing rates on your profile. Whether you are staff from January, a director on a practice arrangement, or resourced per engagement changes how you are paid and what you can commit the firm to. Settle it before the start rather than discovering it at the first invoice run, because a January start with unsettled terms turns the first fortnight into an administrative conversation instead of a commercial one.

What you are doing between now and then is also worth agreeing explicitly rather than leaving to drift, because the two obvious answers are different sizes. Reading this pack and forming a view is one thing. Carrying the Aman engagement, which is already yours, is another. Picking up anything from Part Eight's held list is a third and it would need a separate arrangement.

Separately, there is no business development budget, no sales team, no customer relationship system beyond the platform's own Communications surface, and no agency rate card for the service line you would be selling into the Grandville conversation. None of those is a reason not to start. All of them are things to put in front of Debo as decisions with options rather than as complaints, and the three months before you start are a better time to do that than the three weeks after.

Part Three: The clients, and the growth in each

3.1 The whole book on one page

Seventeen client records, twenty one engagements, of which the ones below are live or recently closed. Read the fee shape column as the structure only; the figures are on the platform.

Client	Where	What we are doing	Status	Fee shape
Arabella Women's Health	Asokoro, Abuja	Diagnostic audit, gate to a management contract	Started 2 Oct, runs to 30 Nov	Fixed fee, then a proposed share of revenue
Medbury Healthcare Group	Lagos and Abuja	Division setup and management, campus conversion, aesthetics venture, launch programme	Division deal closed 21 Sep, setup running	Setup in tranches, monthly support from month six, performance share

Client	Where	What we are doing	Status	Fee shape
Haven Paediatric Centre	GRA Ikeja, Lagos	Turnaround, culture build, then the growth mandate	Active to Jan 2027, instalments outstanding	Fixed fee in instalments
Belfiore Medical	Ikoyi, Lagos	Client records system, then the client experience programme	Active, balance outstanding, second invoice not raised	Fixed fee per phase
Aman HMO	Abuja and Lagos	Lagos provider network growth. **Your practice.**	Active, balance past due	Mobilisation then monthly execution
Osteon Clinics	Amuwo Odofin, Lagos	Organisational audit	Fieldwork done 17 Sep, report due	Carried at our cost, not billed
Havana Specialist Hospital	Lagos	Board governance framework	Complete, paid in full	Fixed fee, board partner concession
House of Refuge, Freedom Foundation	Lekki, Lagos	Management retainer and a fundraising mandate	Running, not on the platform as an engagement	Monthly retainer plus a share of funds raised
Dr Chinwe Kpaduwa	United States and Lagos	Her visit, and the aesthetics venture around her	Planning, she leaves in November	Inside the Medbury structure
Medlyfe Introduces	Lagos	Launch programme, evening on 8 Oct plus an eight week campaign	Active to Dec	Programme fee
Covally, Connexxum, Cureva, Cooked Indoors, Osibil, Doctoora	Lagos	Founder and venture portfolio	Mixed, mostly planning	Internal
Priscilla Specialist Hospital	Ajah, Lagos	Management and digital transformation	Cancelled, relationship intact	Was a large transformation mandate
Consult For Africa	Internal	Firm setup, plus the Maarova and CadreHealth go to market engagements	Active to 15 Dec	Internal

Note that last row, because it is yours. **The Maarova go to market and the CadreHealth go to market are both live internal engagements with an end date of the fifteenth of December 2026.** They exist on the platform as engagements with nobody driving them. That is the clearest possible statement of what this role is for.

3.2 Arabella Women's Health, Asokoro

Dr Chito Nwana leads a women's health facility in Asokoro, Abuja, as both principal and lead clinician, with Tolu as her Chief of Staff. The engagement closed on the twenty eighth of September after negotiation since April, and the audit began today.

Arabella is a new entity with its own registration, succeeding Tabitha Medical Centre, and that succession is the most important fact in the file because it decides what moved across and what did not, including the patient records, the staff contracts and the health insurance panels.

The scope is the diagnostic audit and nothing else, which is Debo's instruction, and it is the sentence to hold on to because the proposals Dr Chito already holds describe a great deal more. Those documents set out a two month foundation phase including a digital marketing strategy, a public relations strategy, a pharmacy inventory audit, a client records system with all patient facing staff trained inside a fortnight, an insurance successor programme, a seconded Head of Quality and Clinical Operations, a reactivation campaign against the Tabitha patient database, and a live dashboard refreshed daily. None of that is being delivered now.

Why this matters commercially to you. That unsold phase is not a problem, it is the pipeline. Every item on it has already been scoped, costed and read by the client, and the forward path is a management contract charged as a share of monthly revenue from month three with a floor in the early months, which against even the conservative revenue case is worth a multiple of the audit over a year. The audit is the qualification round, and every deliverable over the next eight weeks is being read by somebody deciding whether to hand us her business.

So the growth job here is not to sell anything for eight weeks. It is to make sure the audit lands well enough that the management conversation happens, and to have the management proposal ready in a form she can say yes to, as one number a month with no tiers and no itemised invoice.

The one risk to watch is the gap between what she has read and what we are doing, because nobody has written down that the scope is narrower and she will reasonably expect the larger thing. Ezinne owns closing that in writing. If Dr Chito or Tolu raise an item from the unsold list with you, note it and bring it back rather than agreeing to it, and treat it as qualification: the items she asks about unprompted are the ones that will sell.

Everything the client needs sits at consultforafrica.com/ArabellaProject, which is unindexed, live and verified in production. It carries the information request, the uploader, four survey instruments and a week by week view. It has not been sent, because her email address is not recorded anywhere in our systems.

3.3 Medbury Healthcare Group

This is the firm's largest commercial relationship, the group chief executive is Dr Itunu Akinware, and it has four strands. It is also the best worked example in the firm of a small engagement becoming a very large one, so it is worth understanding in detail even though you did not sell it.

The division deal, which closed on the twenty first of September. A fixed setup fee over five months across Lagos and Abuja in four tranches, exclusive of tax and of government fees, with Medbury paying the government fees directly or reimbursing at cost with no mark up. Two chief operating officers are employed and paid by Medbury but **recruited by CFA at its own cost and seconded to CFA reporting to Debo**. A monthly support fee runs from month six for the shared functions and for directing the division toward the board's goals. A performance share on collected net revenue is payable only where the board's revenue goal and its margin goal are both met. The term is twenty four months, reviewed at twelve.

The reporting line is the load bearing clause. Debo flagged during the negotiation that putting the two operating officers on Medbury's payroll makes us dispensable, and the answer was secondment, so we have direction without employment. The memorandum says in terms that the arrangement works only while

that reporting line is observed in practice as well as in writing. If those two people start taking direction from Medbury, we are an invoice.

The payment terms were hardened deliberately, and they are the template Debo now wants on every new retainer. The support fee is payable monthly in advance by the fifth business day of the month it relates to, we invoice by the twenty fifth of the preceding month, both sides put it on a standing payment instruction so it does not depend on anybody approving it monthly, one month is held as a deposit and applied to the final month, and a disputed line does not hold up the undisputed part. When you are shaping a new retainer, the three terms to make sure survive the client's redline are the standing instruction, the held deposit, and paying the undisputed part on the due date.

Lyfe Place Abuja is a four structure residential compound being converted into a private ambulatory medical campus, theatre on the ground floor and family practice above. Our work recovered the building's measured geometry from the as built drawings rather than accepting the brief, and the brief was wrong: the campus is larger than stated, the extra area is all corridor, so the rent roll is flat while the setup costs materially more, and there are eight consulting rooms rather than the one the original model allowed for. Two questions remain open and they change the answer rather than the confidence in it. The wellness catalogue assumes a substantial annual volume of visa medicals, which in Nigeria runs through embassy and international organisation panel physicians, so if Lyfe Place is not on those panels the revenue is unreachable rather than under marketed, and that is one phone call nobody has made. And two of our own documents disagree on sessional fill by a factor approaching two on the largest revenue line on the campus, so neither can be quoted again until they are reconciled.

The aesthetics venture is a jointly held vehicle agreed in August with its own name and board, built around Dr Chinwe Kpaduwa, who practises in the United States and leaves Nigeria in November. There is a separate Lagos setup mandate and a management fee structure of a monthly base plus a share of collections and a share of margin.

Medlyfe Introduces is a themed evening of a fireside chat, a panel and a cocktail reception on the eighth of October under the Medlyfe brand, with an eight week campaign behind it. It is designed as the first edition of a recurring series rather than a launch, so editions two, three and four inherit the format, the identity, the film library and the guest list on Dr Chinwe's 2027 visits. The evening is on the eighth of October, so it happens before you start, and the guest list is the growth asset rather than the evening: it is a room full of exactly the Lagos principals the firm wants, and nobody has planned what we do with it afterwards. It is on the held list in section 8.2 for that reason, because the follow up has to be set up before the evening rather than after it.

A fourth strand, the Alameda conversion clinic, has its own business case and cost base.

Two commercial facts you should know. Itunu said yes in August to the medipark, all the aesthetics vehicles and the conversion centre, plus two recruitment mandates, but **her yes was to the ventures and not to the fee**, which is why locking terms before a team starts is a standing lesson on this account. And she will not override her business unit leads' autonomy, so proposals into Medbury split: group level items go to her, business unit items go to each lead. There are costed proposals already written for the Abuja clinic, corporate wellness, diagnostics growth and functional medicine, each addressed to its own lead. Those are sitting in the documents folder and several have never been pushed.

3.4 Haven Paediatric Centre

A board led paediatric and neonatal turnaround in GRA Ikeja, now in its growth phase, where the founders set the mandate themselves and pushed the timeline to roughly a tripling of monthly revenue against a stated margin floor within six months, led by the neonatal unit into the paediatric high season from November to February.

Two findings from that work come up in conversation often enough to memorise. Most of their two thousand four hundred line tariff was priced at zero and the lines that were priced sat well below market, so a large part of the plan is a repricing in two tranches across the year. And we told them not to host obstetricians' deliveries, because obstetricians keep their own delivery revenue and will not refer into a unit that competes with them, so instead we built an antenatal pathway that pre books neonatal admissions and invited them to tour the unit.

The commercial model here is unusual and worth knowing: the engagement runs on a board partner discount with transparency about the discount, which is the standard, concession, net logic applied to a governance relationship. Three instalments were scheduled for the twenty fifth of August, the twenty third of September and the twenty second of October, and the first two are the overdue part. Kabir Aregbesola is the contact with Usman copied.

The growth in it. Haven is the firm's best proof case for a revenue turnaround and the founders have been through an audit, a culture build and a growth plan with us. They are also a Maarova development stream organisation with five founders assessed. That combination makes them the most likely source of a cleared named reference and the most likely source of a board to board referral, and neither has been asked for.

3.5 Belfiore Medical Aesthetics

Dr Uju Rapu owns and runs an aesthetics clinic on Shoreline Drive, Ikoyi. The client records system with the data protection groundwork behind it is phase one, and a balance on it fell due on the thirtieth of September. A client experience programme for three front desk staff ran on the twenty fifth and twenty sixth of September, priced per head, and that second invoice does not exist on the platform at all, which means nothing is chasing it.

Two things to carry. She has asked for a refund before, on an invoice that was silent on the point, so every Belfiore invoice now carries the terms block stating that fees are non refundable, that a substitution is free and that one reschedule is included. And Belfiore has its own document design rather than our sober audit template, because she is a luxury client and the house template reads wrong for her.

The growth in it. The front desk programme is a five domain client experience curriculum that has now been delivered once and written up as a handbook. It is the closest thing the firm has to a productised, repeatable, low ticket training offer, and it is sellable into every aesthetics clinic, fertility clinic and private practice in Lagos without a diagnostic first. Nobody has tried to sell it twice.

3.6 Aman HMO, which is your practice

Aman Health is a Takaful health company, structured on Islamic cooperative insurance principles, historically northern in its footprint and headquartered in Abuja, which opened a Lagos office in mid 2026 to drive southern growth. The chief executive is Abubakar Hassan Muhammad, who goes by Sadeek Hassan and who sent the brief, and the contact on our client record is Zayyad Abdulrahman, the chief financial officer. They cover around fourteen thousand lives and their brand promise is zero rejected eligible claims.

You are named as practice lead on this engagement in the strategy document, so you know it better than this pack does. What is worth restating is the reframe, because it is the single best piece of commercial thinking the firm has produced this year and it generalises.

The brief asked how to sign three hundred providers. We changed the question to where Aman's enrollees actually live, work and seek care, and which providers are needed there so every member has good access. That turns a volume question into a coverage question and produces a shorter, much more valuable target list. Four things made it the right strategy for them specifically: the seed demand is already knowable from the Lagos branch addresses of the northern corporate accounts they already hold, the Islamic affinity network is a conversion channel email cannot touch, the real conversion lever is payment reliability rather

than persuasion because Lagos providers decline insurers over tariffs and delayed payment rather than over awareness, and CadreHealth lets us seed the network with senior specialists who practise across several hospitals so one relationship extends member access to several facilities at once.

That last point is the one a competitor cannot copy, and it is the first real example in this firm of a product being used to win a consulting mandate. Study it, because doing more of that is most of your job.

The commercial position needs attention. There is a balance outstanding against a due date of the twenty seventh of September, so it is already past due and it is one of the larger receivables in the firm, and **the engagement figure on the platform does not obviously reconcile to the signed commercial**, which was structured as a mobilisation payment plus monthly execution payments. Reconcile before chasing, because chasing the wrong number in writing is hard to walk back.

3.7 Osteon Clinics

Dr Bola Akinola is a United Kingdom trained arthroplasty surgeon in Amuwo Odofin. This started in July as a positioning engagement and became an organisational audit, because the strategy we wrote was a set of promises about the business rather than about the surgeon, and none of them could be executed until the organisation underneath was known.

The thesis we put to him in writing rather than saving for the report is that the practice is bigger than the company: a meaningful share of his income is earned in other people's theatres and invoiced outside Osteon, while Osteon carries a fixed cost base for a surgeon who is frequently elsewhere.

The method is worth knowing because we reuse it and it sells. Ten real cases, picked on the morning of the visit, each followed through a twenty two link chain from first enquiry to current outcome, so every gap found is specific rather than theoretical.

Fieldwork was on the seventeenth of September and the report is due early October. There are still zero survey responses and zero uploads two weeks on, there is no evidence the cover email was ever sent, and his email and phone are not in our records.

The audit is carried at our cost and the documents say nothing about money at all, deliberately, because writing that it is free would price every later workstream against zero. If he raises it, the position is that the audit is groundwork for the work that follows rather than a favour.

The growth in it is larger than the client. There is a strategy in the repository that ties Osteon together with OrthoSurplus, the firm's own implant supply venture, into a single vertically integrated arthroplasty group: anchor surgeons, trained associates, a roaming unit and a packaged offer to hospitals. That is a venture, not an engagement, and it is one of the few places where the firm could own the economics rather than advise on them.

3.8 Havana Specialist Hospital

A Lagos hospital owned by two founding families, both founders now late, with a board of eight split four to four. Debo sits on it as an independent non executive director while CFA was also a paid adviser setting up the board operating model, and that dual position is handled by disclosure and recusal rather than by pretending it does not exist, with the fee approved by the remuneration committee with him recused.

The engagement is complete and every invoice is paid, which makes it the only file in the book with nothing outstanding. The operating partner Afya Care runs the hospital day to day through a joint governance committee, and Favour Olewuenyi is the contact on that side.

The board politics are genuinely sensitive, because one family convened the board and nominated the non executives, so the other family perceives the board as tilted. **Do not discuss this file outside the firm at all, and do not use it as a reference.** It is listed here so you know it exists and know not to touch it.

The one commercial thread worth knowing is that the Afya Care relationship it created is live in the pipeline, covered in Part Four.

3.9 House of Refuge, Freedom Foundation

Pastor Tony Rapu's faith integrated rehabilitation facility in Lekki. The readiness project is complete and we now run management support on a monthly retainer plus a fundraising mandate charged as a share of funds raised, which is a pure contingent fee. There is also a Maarova staff assessment running.

Two things about it matter to you. It is **the firm's only cleared named reference**, as the management partner for a twenty one bed drug and addiction rehabilitation centre in Lekki, and it has already run in our own advertising, so it is the one logo you can actually use. And the fundraising mandate is the firm's first genuinely contingent commercial, which makes it the proof that the model works before you propose it to anybody else.

Neither strand exists on the platform as an engagement, so the monthly retainer is not visible to the invoice run.

3.10 The ventures and the dormant relationships

Kemi Balogun's group is a family group across education, energy, agriculture, healthcare and fashion, for which we delivered a holding company structuring note with five options, a cap table and founders' agreement for the healthcare venture, an instruction brief for her appointed counsel, a one page terms sheet for her clinical co founder, and a rollout plan. The healthcare venture is **ile**, and in September it was inverted: it is a residential eldercare business, and home care plus virtual consultations are the bridge that funds the build, proves the care standard and produces the admissions pipeline that fills the beds on opening day. The structural call that followed is to separate the building from the business, because a single company cap table would make our equity stake a stake in real estate she funded. Status is that the clinical co founder has agreed terms in principle, counsel is instructed and drafting, there is no entity yet, and the waiting list has been live since mid September. For a residential business the waiting list is the occupancy plan rather than a mailing list, so its growth is a real commercial metric and somebody should be watching it.

Priscilla Specialist Hospital in Ajah is where a large transformation mandate was cancelled and the relationship was not. The client record is still active. That is a warm reopening nobody has attempted.

Dennis Ashley, Dr Chima Oti, endoscopy suites in Victoria Island, is under a management arrangement priced as a share of collections with the diagnostic audit carried at our cost. The audit site is live under Mezo branding but there have been zero uploads and zero survey responses, and the promised non disclosure agreement does not exist. Debo has deferred this one, so check with him before touching it.

The family estate is twelve units across three buildings on Banana Island and Osborne Foreshore, which Debo runs facility management for, sitting on the founder side of the platform with link based surfaces for tenants and the ground team. It is not a growth file but it is a worked example of a billing model worth understanding, because a generator run is the billable event and rates are frozen onto each run when it is costed, so moving the rate card never restates a past charge.

Part Four: The pipeline, honestly

4.1 What the platform says, which is the uncomfortable part

Read this section before anything else in the pack.

The pipeline surface at /pipeline holds leads, discovery calls and proposals. As at today it holds **one lead, zero discovery calls and four proposals, all four of them in draft since March and April**.

The one lead came in on the eighteenth of September from a Maarova demo request: Dr Moromoke Ogunko, listed as programme management office at Ifako Ijaiye General Hospital in Lagos, asking about leadership development for around ten leaders on a one to three month timeline, with the inbound message "I have flair for leadership. I need visibility to be able to assume responsibilities of leadership and management." Status is still new. Nobody has replied. It is a Maarova development stream enquiry from a general hospital with a stated leader count and a stated timeline, which is about as qualified as an inbound lead gets, and it has been sitting for a fortnight.

The four draft proposals are for quality and safety auditor recruitment, a SafeCare healthcare quality and patient safety transformation, a healthcare risk management framework, and greenfield healthcare financial modelling and commissioning. Two of them are SafeCare. All were raised in March and April and none was ever sent from the platform, even where a real document went out by email.

So the firm's recorded pipeline is one untouched lead and four six month old drafts, against seventeen clients and twenty one engagements. The pipeline is not thin because demand is thin. It is thin because the pipeline has never been anybody's job, and it becomes yours in January. What happens to it between now and then is the first thing to settle with Debo, because three more months of nobody owning it is how the one open lead becomes unanswerable.

4.2 The real pipeline, which lives in Debo's head

These are live or warm and none of them is on the pipeline surface. This list is the single most valuable page in the pack and the first thing to do with it is get it onto the platform, because a prospect in a document is not a prospect anybody can chase.

Duchess International Hospital, a hundred bed hospital in Ikeja GRA, contact Mrs Ladipo. The proposal, an HR prospectus and a leadership assessment invitation have all gone to her, the fee is payable in three parts, and the scope moved the clinical element to an enterprise tier after the twenty third of September meeting. They are already set up as a Maarova development stream organisation. This is the furthest advanced prospect in the firm and it is waiting on a decision.

Clearview Fertility and Hospital, Dr Kunle Ajayi, an obstetrician and gynaecologist in Lekki. An inbound fertility medical tourism thesis where the argument is that the hospital is what turns a clinic into a destination. There is an Osteon shaped strategy and a discovery instrument of more than a hundred and twenty questions already built. Debo is moving this one toward a performance based commercial.

Iwosan Lagoon and the IASO Medipark, Fola Laoye, whom Debo knows from the Doctoora raise that led to his Eti Osa isolation centre role. A warm approach timed around the opening of a hundred and forty bed facility, so the window is the year either side of that opening.

Afya Care, Tosin Runsewe, who knew Debo at Evercare and Cedarcrest and is now a board chairman, having conceded the Oakwood chief executive seat. The target is a fractional group level function that CFA can live inside. Note he is also the operating partner at Havana, so this relationship has two faces and the Havana side is sensitive.

Solace Healthcare, Games Village, Abuja. Dr Oby Clems-Anunwa is a consultant family physician at National Hospital Abuja converting four to five built residential units into a health facility, phased as day case ambulatory, then inpatient at half capacity, then a centre of excellence. We delivered the advisory and proposal on the fourteenth of September and there has been no decision. The finding that drives the document is that the converted site alone does not reach her target inside two years while the same site plus a small leased diagnostic and clinic spoke, opened near the end of year two, does, so her phase three ambition contains a phase one decision. The positioning matters: the African Medical Centre of Excellence is under twenty minutes away and does the entire cancer pathway including radiotherapy, priced against Chennai and Istanbul, so Solace sits underneath it at roughly half the flagship tariff, and we never frame

Solace as a competitor to that centre. The fee is anchored high and built from resourced days plus a programme rate over a twenty seven month programme, and there is a private negotiation guide with the floor and the concession ladder which is never sent to a client.

Grandville Medical Group. A cold discovery call with two marketing businesses inside one group needing opposite strategies. A deck has been built. The problem is that the agency rate card does not exist yet, so the anchors in that deck need Debo's sign off before they are quoted. That is a decision to force rather than a document to write.

Pearl Oncology Specialist Hospital, Lekki Phase 1. Dr Omolola Salako is founder and chief medical director, Dr Adaorah Enyi is director of research and innovation and our introducer, and Mrs Oluwakemi Ayorinde is the HR manager and day to day contact. Their need is medical officers for the oncology facility. As at the middle of May, Kemi had asked how to proceed with the engagement and next steps, and was awaiting our pilot proposal. That has been outstanding for four and a half months. Pearl is a strategic logo: a specialist oncology hospital with a credentialed research director and a press profile, whose peers would notice.

Osiris Health, chief executive Ayoola Oladipupo, founder led and a single decision maker. A multi site dialysis operator managing Life Support Medical Centre, one of Nigeria's earliest dialysis providers, now expanding with a collaboration at R Jolad Hospital Gbagada. Their need was four distinct cadres across three sites simultaneously: an operations manager in Lagos, a consultant nephrologist across Lagos, Ibadan and Warri, medical officers in Ibadan and Warri, and dialysis nurses in Ibadan and Warri. **Their second need is partnership.** Ayoola wants to be both a client and a partner and said he is sold on what we are building. That conversation has also been open since May. Handled properly it is the template for the next ten hospital partners, and the right structure is two parallel agreements rather than one, with equity explicitly parked for a separate meeting rather than embedded in a recruitment agreement.

SafeCare and PharmAccess Foundation. Two of the four stale draft proposals are theirs. They run large quality and patient safety programmes and need specialist healthcare consultants without holding a bench, which is exactly the partner firm proposition in section 5.7.

HealthStack Africa. Their founder asked about collaborating to hire and deploy community management agents across Nigeria as records and digitisation advocates providing system agnostic support, meaning they help a facility adopt any system rather than pushing one. We have the agent channel built, the workforce platform to source from, and a digital health service line. It needs a discovery call before any structure is proposed, and that call has not happened since April.

The Healthcare Federation of Nigeria is the private sector umbrella body and its President is Njide Ndili. Debo is a member rather than in its leadership. In July he drafted a policy brief on the medical internship placement bottleneck for the federation's leadership to adopt and send to Professor Iziaq Salako, Minister of State for Health, written in the federation's collective voice so leadership could put it on their own letterhead. The anchor statistic is that roughly two thousand of about six thousand annual medical graduates are stranded against an accredited capacity of around four thousand, which the Medical and Dental Council's Registrar told the Senate, and the Council has already said publicly that the fix is state and private hospitals, so the door is open rather than needing to be forced. Nobody knows whether it ever reached the Minister. Institutional correspondence of this kind is a category of work in its own right, it moves on other people's timelines, and it needs the same chase discipline as a client.

4.3 What the pipeline needs structurally

Three things, none of which is a tool purchase.

A record that exists. Every name in 4.2 onto the pipeline surface with a stage, a next action and a date. Until that happens the Thursday sweep has nothing to sweep and Ezinne cannot help you.

Qualification that is written down. The platform has fields for qualification score and notes and they are empty on the one lead we hold. Whatever your qualification standard is, write it once and apply it, because an unqualified pipeline is just a list of people who were once polite to us.

A decision on the four drafts. Either send them, rewrite them, or close them with a reason. Six month old drafts in a pipeline make every forecast from that pipeline meaningless.

Part Five: The products, which are yours

5.1 Why the products exist, and the pattern across all of them

The firm built products because the same three things break in every institution it is called into, and recommending a fix nobody in this market can buy is a waste of a diagnostic. Maarova is the leadership answer, CadreHealth is the workforce answer, Mezo is the practice economics answer.

There is one pattern across all three and you should expect to find it everywhere. **The product works, the data is real, and almost nobody was ever told.** The Mezo measurement is the cleanest statement of it and it is worth reading twice: conversion is about sixty eight per cent among doctors who were actually emailed that they had a place, and zero out of nine hundred and seventy among those who were not. Nobody has ever found it on their own.

So before you plan a campaign for any of these, the first question is not what the product should do next. It is who already has a reason to want it and has never been told it exists.

One rule applies across every one of them, without exception. **Never name our platforms in client body copy.** Describe what is done instead, so a client document says staff will be trained on a secure client record system, not that we are deploying a named product. The only exception is an appendix a client has specifically asked for.

5.2 CadreHealth

What it is. A career intelligence and workforce platform for Nigerian healthcare professionals, deliberately not a job board. It is a professional identity and intelligence layer that helps a healthcare worker earn more, prove their credentials and steer their own career. The tagline is "your career, your call." The intended domain is oncadre.com and it is not yet registered, which is an open risk, along with the defensive domains cadrehealth.africa, cadrehealth.ng and cadre.africa.

The audience. All sixteen healthcare cadres, not only doctors: medicine, dentistry, nursing, midwifery, pharmacy, medical laboratory science, radiography and imaging, rehabilitation therapy, optometry, community health, environmental health, nutrition and dietetics, psychology and social work, public health, health administration, biomedical engineering. Core band is roughly twenty five to forty five, early to mid career. Secondary is the diaspora. Tertiary is medical students choosing where to do housemanship, who have the highest search intent of any group.

The audience insight that drives everything. Around eighty per cent of Nigerian healthcare workers intend to emigrate, and more income makes emigration easier and faster. So the platform has to deliver immediate earning value, and migration must be framed as a choice the professional is making, never as an exit we are helping them take or a betrayal we are talking them out of.

The six pillars. The Career Readiness Score is the viral front door: free, no account, fifteen questions, producing a domestic employability percentage plus readiness percentages for the United Kingdom, United States, Canada and the Gulf, with a shareable card. Salary Intelligence runs on give to get, where a two minute salary survey unlocks the salary map, cadre specific, facility specific and city specific, with an annual report as the press and backlink play. Hospital Reviews are anonymous and verified across eight dimensions, which are compensation, pay timeliness, workload, equipment, management, safety, training

and accommodation, with give to get on the review text and ratings always visible. The Credential Wallet tracks licence and regulatory status with reminders, holds a qualification vault and a continuing professional development tracker with gap analysis, and shows a readiness dashboard by destination. The Locum Marketplace is phase two. The Verified Anonymous Community verifies a licence number against the regulator's register then cryptographically separates it from activity, so people can speak freely about their own facility. Alongside those sit a career advisor, a curriculum vitae builder, a job board, mentorship, migration pathways and exam guides.

The state of it today, which decides every campaign you plan. Read these numbers before you promise anybody a list.

	As at 2 October 2026
Professional records held	10,248
Hold a password, so genuinely registered	1,059
Have ever logged in	957
Eligible for Mezo, being medicine or dentistry and registered	1,024
Career readiness assessments completed	88
Career reports generated	30
Salary reports submitted	124
Facility reviews submitted	**1**
Newsletter subscribers	11
Subscriptions, of which free	85, of which 84
Subscriptions actually paid for	**1**
Mentor profiles created	5
Mentorships ever started	**0**
Outreach records	10,225
Logged outbound messages	20,106, every one of them email

The register has grown from roughly six hundred and sixty logged in users at the start of September to nine hundred and fifty seven, and five hundred and fifty six of them logged in during September alone, so the base is moving fast and the re engagement campaign is running, with the most recent send on the first of October. Outreach now records eight hundred and thirty seven converted against ten thousand two hundred and twenty five records, which is a little over eight per cent, up from four per cent at the end of August. That is the one product line in the firm with genuine momentum.

The three things that constrain any send, and all three are real. A hard bounce rate of eleven per cent was recorded across roughly nineteen thousand sends because the mail provider's bounce webhook had never been configured, so nothing was suppressed. The webhook is live now and suppressions have gone from zero to one thousand two hundred and forty, but the historical back fill has still not run, which means a little under a thousand known dead addresses remain live targets in the queue. The imported name data is split wrong, with the title in the first name field and the middle name in the last name field, so around twenty

eight per cent of greetings built as doctor plus last name are wrong, and there is a helper in the codebase that exists specifically to get a salutation right. And the WhatsApp channel has never been used: the message table holds twenty thousand rows and every single one is email, so WhatsApp needs number enrichment plus template approval before it is an option at all.

The employer side, rebuilt in September and completely unsold. The diagnosis that drove the rebuild is a good lesson in how a product can be broken invisibly: employer search returned thirty people out of ten thousand because it hard filtered on an availability field no member could ever set. The rebuild is search first rather than post first, on the argument that a Lagos hospital can already post free on LinkedIn and in association groups, but what it cannot do is find the eleven paediatric cardiologists and know which are real. So the register leads and proof comes before commitment. Availability now ranks rather than filters, contact is consent gated so search never carries an email or phone, and an admin verifies employers before they can approach anybody.

The state of it is five employer organisations, four employer accounts, **none verified, none ever logged in**, seven open recruitment mandates, twenty two candidate matches of which every single one came from a candidate applying and not one from us sourcing, zero contact requests and zero shortlists ever created. The seven open mandates are an ecosystem manager, a head of operations for a clinic network, a business manager for supply chain, a chief operating officer for occupational health, a business development manager for medical procurement and healthcare infrastructure, an independent sales partner, and a pharmacist.

So the employer side is a built product with no customers, and the four accounts that do exist are Hartwell Hospital, TowerCare Pharmacy and Novessence Aesthetic Clinic, which was registered twice seventy four seconds apart. Getting those four verified and logged in is a month one job and the cheapest possible test of whether the proposition lands.

Revenue today and the model in full. One person has ever paid for a subscription. He is Dr Francis Korie, a diaspora paediatrician in North Carolina chasing a medical science liaison role, who converted himself off a cold list, and he is the single best piece of evidence the firm has about what a paying member actually wants. He is also owed something: he answered the same intake questions four times because of a bug in the advisor history and left with nothing, and the agreed action is to send him the thing he was promised and never received, which is a professional profile and a United States format resume, rather than an apology. That has not been done. Debo wants to see the copy before anything sends.

The full revenue model is placement fees on recruitment, locum margins, employer subscriptions, insurance distribution, earned wage access fees, workforce intelligence sold to ministries and multilaterals, continuing professional development content partnerships, and international recruitment referrals. Of those, exactly one has ever produced money, and it is the subscription that produced the least.

Competition. The closest digital competitor is ProLocums and it is early stage. The actual incumbent is WhatsApp groups. Nobody does salary intelligence, hospital reviews or credential management for Nigerian healthcare, which is both the opportunity and the warning, because there may be a reason.

What is most sellable here, in order. The recruitment service, because it has two warm prospects already asking and a pricing playbook already written. The employer subscription, because it is cheap enough to be an impulse and it seeds the recruitment conversation. And workforce intelligence, because ten thousand records with real salary data is a dataset a ministry, a federation or a multilateral will pay for and nobody has ever offered it to one.

5.3 Mezo Health, and the deadline that falls before you start

What it is. A verified specialist network and a hospital operating stack. A doctor consults at network facilities or by teleconsultation, and Mezo fills the clinic, runs the schedule and collects the payment. It answers a

specific and very common problem: a good doctor who wants a private practice does not want to become a landlord, a receptionist, a billing clerk and a claims chaser in order to have one.

Status. Mezo has no separate company registration, so it runs as a CFA product and service offering, and in client facing material it is named as **CFA's facility operations practice**. It has its own codebase, its own database, its own authentication, and it lives at mezohealth.com.

What it does for a doctor. Facility connections so a doctor can consult at rooms they do not own, schedule and clinic management, payment collection, and claims processing with the health plans. On the growth plan there is advance factoring of up to eighty per cent against pending claims, and that is the line that matters most in any conversation, because the reason Nigerian private practice strangles good doctors is not that the money is not real, it is that it arrives late.

Plans. Starter, Growth and Diaspora. The diaspora plan exists because a Nigerian specialist abroad can hold a practice presence here without relocating, which also makes it the natural product to put in front of the Doctors Foundation for Care network.

What it does for a facility. Room and session utilisation, through a model of facilities, rooms, access rights, group room contracts and visit blocks. An underused consulting room is pure margin once it is filled, which is the whole argument to a facility owner, and it is the same argument that sits underneath the Lyfe Place campus.

Patient side. A membership product.

Eligibility. Mezo runs on the medical and dental register, so places are open to medicine and dentistry only, which covers all but a few dozen of the CadreHealth records, so the constraint costs almost nothing. The offer is hidden from ineligible cadres rather than shown and then refused.

The positioning rule, and this is the important part. Sell Mezo as a private practice opportunity and sell it aspirationally. The line is "your own practice, without building one." It is never "claim your account", never "sign up", never "register". The audience is senior clinicians who have been pitched by everybody, and consumer application language reads to them as junior. The senior medic copy rules in section 7.4 apply to Mezo more than to anything else the firm writes.

The state of it, measured today.

	As at 2 October 2026
Doctors on Mezo	989
Places actually claimed	**22**
Open, unclaimed stubs	967
Expired so far	0
First claim links expire	**11 October 2026**
Last claim links expire	27 October 2026
Mezo eligible registered doctors on CadreHealth	1,024

Three things follow from that table and they are the most time critical items in this pack.

The first is the measurement, which settles an argument. Every one of the doctors who has claimed a place came from the small group who were actually emailed. Cross matching the claimers against everyone ever sent a claim email showed that all of them came from the twenty eight members who went through the

survey path and were told, and **zero of the nine hundred and seventy who were never told have claimed anything.** So conversion is roughly sixty eight per cent when a doctor is told they have a place and zero when they are not. The low headline rate is not a product problem or a positioning problem. It is an artefact of never running the campaign.

The second is the deadline. The earliest nine hundred and seventy claim links die on the eleventh of October, which is before your start date, and re seeding afterwards simply reproduces the same silence. There is a cron that reissues them, so this is recoverable rather than fatal, but the decision belongs to October and not to January, which is why it is the first line on the held list in section 8.2. Whatever is decided, read the position off the platform in January rather than assuming this paragraph still describes it.

The third is a gap. CadreHealth now holds one thousand and twenty four Mezo eligible registered doctors against Mezo's nine hundred and eighty nine, so roughly thirty five newly registered doctors have no place open at all. The backfill is a point in time sweep and needs re running before any send.

One important operational caution. The seeding cron is deliberately silent: it writes no interest record and sends no email, because a separate retry cron mails anyone holding a claim link with no send stamp and would otherwise blast a thousand unsolicited emails on its next hourly run. So anybody running a Mezo campaign needs to understand that sequence before touching it, and the engineering side needs to be in the room when it is planned.

What this is worth. If the sixty eight per cent figure holds at even a fraction of its level across nine hundred and seventy doctors, Mezo goes from a product with twenty two users to a network with several hundred practising specialists, and that network is simultaneously the supply side of the Aman style payer strategy, the fill for every underused consulting room in a campus like Lyfe Place, and the single most defensible asset the firm owns. It is also the clearest example in the firm of value sitting one email away from being realised. Mezo has no brand guide at all and runs on its own product styling, so if a campaign is approved, the brand definition needs scoping before it starts.

5.4 Maarova

What it is. A psychometric assessment and coaching platform for healthcare leaders, calibrated to the African context rather than translated into it. The tagline is "assess, develop, and retain Africa's healthcare leaders." The buyer is almost never the person being assessed, which is the most important commercial fact about it: you are selling to hospital groups, training hospitals, health system foundations, boards and HR directors who are hiring or developing frontline, middle and executive leaders.

The three streams, and every Maarova organisation is set up on one, which decides which modules run. Recruitment is assessment as part of a hiring decision, sold as "hire with science, not gut feel." Development is assessment as the opening of a coaching relationship, sold as "assess, coach, measure, repeat." Intelligence is assessment across a whole leadership population to see the landscape, sold as "see your entire leadership landscape."

The six modules. Behavioural style, values, emotional intelligence, clinical leadership transition, culture, and a 360 feedback module. The clinical leadership module is deliberately restricted to clinical professionals so a non clinical leader is not penalised for not having it, and the 360 is decoupled from the core because it depends on external raters and takes days or weeks. Results appear progressively as each module completes, so a participant gets value immediately rather than at the end.

The methodology, which is the strongest and most underused sales asset in the firm. This is what distinguishes Maarova from every off the shelf instrument in this market, and almost none of it has ever been published.

The Hofstede scores that most consultancies still quote for Nigeria come from employee surveys conducted between 1967 and 1973, and the Nigeria figure was derived from a three country West African aggregate rather than a Nigeria specific sample, so Maarova treats Hofstede as background literature rather than as a load bearing framework. The integrity backbone is HEXACO, whose honesty and humility factor outperforms every Big Five dimension as a predictor of counterproductive work behaviour and which has been validated in Nigerian samples. That is combined with the South African Personality Inventory, the most rigorous indigenously developed African personality assessment in existence, built across all eleven official South African languages from how native speakers describe personality rather than by translating a Western inventory, and whose integrity factor weights fairness and resistance to discrimination, which fits the African pattern of integrity failure better than a greed weighted Western construct does. Response bias is handled through the practice versus values gap from the GLOBE study, which measured each dimension twice, once as what people do and once as what people think should happen, because the size of that gap predicts which items will pull answers toward the aspirational pole. The response format direction is quasi ipsative forced choice scored with Thurstonian item response theory, which is materially harder to fake than a rating scale and supports valid comparison between candidates.

The coaching differentiator is the **Ubuntu framework**, where goals are framed around collective impact rather than individual achievement and the coaching question is how your leadership serves your community, with the coach positioned as a thought partner rather than an authority so that power distance is respected rather than ignored. No validated Ubuntu leadership instrument exists anywhere, which is an opportunity and it is ours to take.

There is coaching evidence worth quoting in a room. The single strongest predictor of coaching outcomes is the working alliance, which is the quality of the relationship, at a correlation of roughly point four one, and that is more predictive than credentials, session count or methodology. Mayo Clinic found six coaching sessions significantly reduced physician emotional exhaustion. Industry return on investment is reported at five to eight times when measured on productivity and retention.

The coaching product shape. One to one is twelve sessions over six months, biweekly, by video plus messaging. Group coaching is four to six peer leaders meeting monthly over three to six months. On site workshops run half or full day for ten to thirty leaders. Leadership retreats run two to three days offsite for ten to twenty. An annual conference for fifty to two hundred is the thought leadership vehicle. Coaches are sourced from the certified coaching bodies across Nigeria, South Africa and Kenya, with a target acceptance rate of ten to fifteen per cent as a quality signal, and a fifteen minute chemistry call before any engagement starts, which is non negotiable.

The pricing argument, without the numbers. The anchors are Lagos Business School's Advanced Management Programme and its Chief Executive Programme, and Harvard's global health leaders programme. Against those, an enterprise cohort of ten leaders costs a hospital about the same as sending one person to Lagos Business School, and an assessment on its own costs less than a conference registration. That comparison is the sales argument and it is a strong one. The rate card itself is in the pricing strategy note and carries a markup applied in May to absorb the Founding Circle discount, with coaches receiving sixty five to seventy per cent of the coaching component.

The state of it today.

	As at 2 October 2026
Organisations set up	26, being 20 recruitment and 6 development
Users	67

	As at 2 October 2026
Assessment sessions	50, of which 29 completed, 13 in progress, 8 not started
Reports generated	26
360 requests created	28, with 40 rater invites sent
Coaches onboarded	4
Coaching matches ever made	**0**
Coaching sessions ever held	**0**
Founding Circle applications	27, of which 26 approved

So the assessment side works and the coaching side, which is where the money is, has never run once. Four coaches are onboarded, twenty six reports have been generated and handed to somebody, and not a single one has converted into a coaching relationship. That is the largest single unclaimed revenue line in the firm and it needs no new build, because the funnel from report to coaching exists and has simply never been worked.

The twenty six organisations are mostly recruitment stream, including Genesis Specialist Hospital in Ikeja GRA, Hisamitsu, Xcene Research and a federal medical centre, and the development stream holds Duchess International Hospital, Haven Paediatric Centre and House of Refuge. Those three development organisations are live clients with completed reports, which makes them the warmest coaching prospects in existence.

What is not yet true, and must not be claimed. A diagnostic in May 2026 on nineteen completed sessions found four things. The emotional intelligence scale is not discriminating, with a median of ninety five and a floor of eighty, so it is measuring social desirability as much as emotional skill. The 360 funnel collapses, with seventy two per cent of subjects who created a request never sending a single rater invite. There is no integrity construct in the current six modules at all, so integrity vetting is not a claim the instrument supports. And response time is not captured on any item, which removes the cheapest available check on careless responding. These are being fixed as version one updates, and they are framed as version one updates rather than as a version two, deliberately. Until they are fixed, **do not write anything that claims predictive validity, normative percentiles or integrity measurement.**

Voice rules. Science, not horoscope: lean on validity, reliability and predictive language, and never describe results as personality types. Coach led, not algorithmic: every output points to a human coach, because the platform supports the relationship rather than replacing it. And measured language for leaders, because the audience is senior consultants and chief medical directors. Say psychometric science meets African healthcare, and paired with a certified coach from our African network. Avoid personality test, find your type, and anything that reads as automated insight.

The one lead the firm holds is a Maarova lead, which is worth noticing. The demo request route works. Nobody has ever answered one.

5.5 The HR and workforce practice

Oludayo Olugbamigbe is practice lead, joining from the Nigerian Exchange Group where she is group talent management specialist and HR business partner, with a master's in industrial relations and personnel management from Lagos and a psychology degree before it. At the exchange she took leadership pipeline readiness up by a quarter, classified ninety five per cent of the workforce under her own talent identification framework, brought attrition down to five per cent and raised application completion by sixty five per cent.

She is a psychologist before an HR practitioner, which is why assessment, coaching and culture measurement sit inside the practice rather than being bought in.

Two cautions on her credentials, both for internal use. Her curriculum vitae writes one certification issuer as a transposition of the correct initials, and it lists an incomplete medical qualification which is deliberately left off client documents because it invites a question that undersells her.

Six services, sold in this order: recruitment and executive search, leadership assessment, coaching and development, culture and engagement measurement, salary benchmarking and workforce planning, and interim and fractional leadership.

Recruitment pricing moved on the twenty third of September and you need the current position rather than the memo's. Debo set it at eight to fifteen per cent of first year gross, down from fifteen to twenty, with the lower end for nursing and allied batches and the upper end for a scarce senior specialist, and a three to six month replacement guarantee. The modal Nigerian market rate is twenty per cent, so this is a deep undercut and the economics rely on batch volume or on platform sourcing rather than on effort. That is a commercial position you should test, because the earlier memo argued explicitly against undercutting on the grounds that we compete on platform leverage rather than price, and an eight per cent floor on a nursing batch is thin.

The rest of the memo is still good and worth reading in full, particularly the guardrails. Hold firm on payment terms, on the withdrawal fee, and on replacement covering voluntary resignation only, because we cannot underwrite poor people management on the client's side. Concede on payment timing for slow institutional clients, on the retainer where a client signs a multiple role minimum, and on the service level where a role is genuinely niche. Walk away from pay only on survival clauses, from exclusivity across cadres a client is not actively hiring for, and from any client asking us to suppress the readiness score, the salary data or the review surface from the candidate side of the platform, because the platform is not negotiable.

The culture thesis is the spine of the practice deck and it is Debo's own, so learn it. The presenting cause is always a thing and the reason is never the thing. Three things make a culture and none of them is a poster: the routines people follow, the incentives you actually pay, and whether it is safe to say something. The line that lands in a room is that the missing item is the symptom, while the absent routine and the unaligned incentive are the disease.

The staff application built in late September gives the practice something to demonstrate rather than describe. It carries shifts, availability, earnings, payslips, recognition, anonymous pulse surveys and onboarding. Two design decisions in it are worth knowing because they are the kind of detail that wins a sceptical HR director: a pulse response carries no staff identifier at all, using a one way hash of the check and the person so that a second response is blocked without the first becoming attributable, and payslips are filtered to issued and paid only so nothing in draft can reach a worker's phone. It is built and typechecking but not deployed, so a demo needs an environment stood up first.

5.6 The marketing agency

The agency is a service line and a real business, run by Nimi, offering positioning, brand, digital and patient acquisition for healthcare providers. It is also the least commercially defined thing in the firm, because **there is no rate card**, which is currently blocking the Grandville conversation.

What exists is the operating brief, which sets out what the agency owns, the service levels it commits to, a three level KPI framework separating agency commercial performance from account delivery performance from our own brands, quality gates described as non negotiable, and a set of standard engagement shapes intended to productise delivery. The pricing section sets out the logic, which is a published rate card, retainer floors, and scope defined tightly enough that a change is visibly a change. The logic is there. The numbers are not.

So the agency is a decision waiting on Debo rather than a product waiting on a market, and forcing that decision is a month two item, because you cannot pitch Grandville or anybody else without it. It is also the one thing in this pack that could usefully be settled before January, since it needs a conversation rather than a campaign.

One honest point from the brief worth carrying into any agency pitch: in a referral led business, marketing is not judged on inbound volume. It is judged on making the firm findable, giving a referrer something to send, shortening the time from first conversation to signature, and building the standing that makes an owner take the call. Sell the agency on the same basis, because a healthcare provider who expects leads from a brand engagement will be disappointed by a good one.

5.7 The channels that were built and never used

Three of these exist as working software with nobody in them. Each one is a distribution strategy somebody designed, built and then did not launch, which makes them the cheapest growth available to you.

The agent channel, at /agent, is a commission based field sales system with opportunity tracking, a deal pipeline and payouts. It holds four approved sales agents, one opportunity, which is an independent sales partner role for Iyewo Health Plans posted in April, **zero deals and zero commissions ever**. Four people were approved as agents five months ago and given one thing to sell. This is a ready made route to market for the products and for the lower ticket services, and it is sitting idle.

The partner firm portal is designed for external consulting and advisory firms to engage and deploy CFA consultants on their own projects, with CFA as employer of record rather than broker, managing contracting, billing and quality, and earning a margin on consultant rates. The models exist on the platform. **There are zero partner firms.** The trigger for the idea was exactly the SafeCare and PharmAccess situation: organisations running large quality and patient safety programmes who need specialist healthcare consultants and do not hold a bench. We hold a bench of sixty nine consultants. Two stale draft proposals in our own pipeline are to one of those organisations. That is a proposition with a prospect already attached.

HealthStack Africa asked in April about collaborating to hire and deploy community management agents across Nigeria as system agnostic records and digitisation advocates. There are three possible structures and all of them use things we already have: listing their advocate opportunity on the agent channel so agents earn per facility onboarded, creating a digital health advocate category on the workforce platform and charging for a verified pool, or fronting the consultancy ourselves with their agents as the embedded delivery layer. The open questions are who employs the agents, who funds it, what the geographic priority is, how compensation works and who certifies the agents, and none of them can be answered without a discovery call. That call has not happened in five months.

5.8 The product growth engagements that already exist

Worth repeating from Part Three because it is easy to miss. **The Maarova go to market and the CadreHealth go to market are live engagements on the platform under the firm's own client record, both ending on the fifteenth of December 2026.** Somebody set them up as real engagements with real dates. Nobody has been driving them. They are the formal container for most of what Part Five describes, and the first useful thing you can do with them is give each one a plan and a weekly number.

Part Six: The growth arithmetic

6.1 What this part is for

You will be offered more opportunities in your first month than you can take, almost all of them real, and the firm has no way of ranking them. This is an attempt at one. It is Debo's view rather than a model, so argue with it, and arguing with it now is more useful than agreeing with it in January.

The ranking is written as at October, so the top two lines will have been resolved one way or another before you arrive. Read the order as the reasoning rather than as the queue: what makes a lever worth pulling here is evidence that the thing already works and that the only missing piece is somebody telling people about it.

6.2 The levers, roughly ranked by what they would produce against what they cost

Tell the nine hundred and sixty seven doctors they have a Mezo place, before the eleventh of October. The highest ratio of value to effort in the firm, because the conversion evidence already exists, the accounts already exist, and the only missing piece is an email. It needs the engineering side in the room because of the cron sequencing, and it needs the senior medic copy rules followed exactly.

Convert the twenty six Maarova reports into coaching. Four coaches are onboarded, zero coaching has ever happened, the rate card exists, and three of the organisations holding completed reports are live clients. No build required.

Answer the Pearl and Osiris recruitment conversations. Both have been asking since May, both have named needs across multiple roles, a pricing playbook exists for both, and Osiris wants a partnership on top. The posture and the questions to ask are already written.

Get Duchess over the line. The furthest advanced prospect in the firm, with the proposal, the HR prospectus and the assessment invitation all delivered, waiting on a decision.

Reopen Priscilla and chase Solace. One is a cancelled mandate with an intact relationship, the other is a delivered proposal with no response since the fourteenth of September. Both are cheap.

Sell the Belfiore front desk programme a second time. It is delivered, written up as a handbook, low ticket, needs no diagnostic, and every aesthetics and fertility clinic in Lagos is a prospect.

Put the four agents to work and open the partner firm channel. Built, staffed in one case, and empty.

Settle the agency rate card and pitch Grandville. Blocked on one decision from Debo.

Offer workforce intelligence to an institution. Ten thousand records with real salary data across sixteen cadres is a dataset a federation, a ministry or a multilateral will pay for, and nobody has ever offered it to one. This is the slowest of these and possibly the largest.

6.3 What to measure, and what not to

Four numbers would tell Debo more than any report, and the figures beside them are today's rather than January's. The number of live qualified opportunities on the pipeline surface, which is currently one. The number of cleared named references, which is currently one. The number of product users who were actually told about the product they are eligible for. And the time from first conversation to signature, which nobody has ever measured here.

What not to measure is inbound lead volume, because in a referral led business it rewards the wrong activity and it will make a good quarter look bad.

Part Seven: How we sell, and the rules

7.1 The methods we reuse, which are also the pitch

These have names inside the firm and each one is both a delivery method and a reason to be hired. Knowing them lets you describe how we work in a room without promising a deliverable.

The four stream diagnostic and **the ten case trace**, where ten real cases are picked on the morning of a visit and each followed through a twenty two link chain from first enquiry to current outcome, so that every gap found is specific rather than theoretical.

The forced tradeoff leadership survey, where respondents allocate a fixed number of points rather than rating everything highly, which is how you get a real answer out of a leadership team.

Measured geometry, which is the habit of recovering the facts from source rather than accepting the brief. It found that the Lyfe Place campus was substantially larger than the brief said and that the extra area was all corridor, which changed the entire financial case.

Decisions with options, not recommendations, so a board chooses rather than being told.

The options note, which is the stage one document described in section 1.5.

Standard, concession, net, which is how every fee is presented.

Give to get, which is how both the salary map and the hospital reviews are unlocked, and which is a genuine design principle rather than a growth hack.

One link, not six attachments. A client gets a single unindexed page carrying everything they need. Two are live, for Arabella and for Osteon. This is a surprisingly strong differentiator in a market where everybody else sends a zip file.

Verify the destination before you send, because a live page can sit in front of a dead endpoint and a success code can sit in front of a parser that read nothing.

Check every claim of absence. When somebody tells you a thing does not exist, verify it, because in this firm's experience the thing usually exists and nobody looked.

7.2 The proof ladder, and why you are short of proof

Published proof is what a referral led business runs on, because the person who was told about us will look us up, and the person doing the telling needs something to send.

What exists is a library of ten anonymised case studies, written up and cleared for use in anonymised form, covering a facility whose revenue record turned out to be a nursing handover sheet, a surgeon who blamed his postcode, a building two hundred square metres bigger than its brief, a chief executive who would not override her own managers, a board of eight split four to four, a phase three decision that had to be taken in phase one, counting providers when the number that mattered was coverage, a venture that had its sequence backwards, an audit of our own instrument, and a policy brief on the housemanship bottleneck. **Not one of them has been published.** Three insights articles drawn from them would change how the firm reads to anybody who looks us up.

What does not exist is named references, where there is exactly one cleared. Getting two or three more is the highest value business development task available and the moment to ask a client is when an engagement is going well, not months later.

7.3 Confidentiality and the anonymisation standard

When our work is described publicly, a client is identified by sector, setting, scale and stage, never by name, owner, district, or any combination of details that would identify them in a market this small. Ratios may be published and absolute figures tied to an identifiable client may not.

Two files are excluded from external use absolutely, which are Haven's clinical incident material and Havana's family politics. The Havana board situation in particular is not to be discussed outside the firm at all.

7.4 Writing to senior clinicians

There is a separate set of rules for outreach to consultants, emeritus physicians, retired clinicians and the diaspora, and it is worth reading before your first campaign rather than after it. The principle underneath all of them is that these are people who have been pitched by everybody and who read application language as

junior.

That is why Mezo is a private practice opportunity rather than an account to claim, why CadreHealth talks to the cadre rather than to the system, and why migration is framed as a choice the professional is making rather than as an exit we are facilitating. Match the copy to the receipts: we earn trust with real salary numbers and real reviews, so aspirational copy without the data behind it is worse than nothing with this audience.

7.5 How we write, and the things that read as generated

Debo rejected a draft in his own name in September with the note that it read as machine written, and the content had been right. The problem was the furniture. Avoid all six of these: three paragraphs opening with matched constructions, the short pivot sentence where a long line is followed by a four word correction, an epigram at the end of every paragraph because one aphorism is a voice and four is a machine, a line commenting on the document itself, paragraphs all the same length, and enumerating in prose with first, second and third.

What works instead is opening with what was actually done rather than a framing sentence, varying paragraph length hard, letting a blunt opinion sit without a line explaining it, and cutting about a third of the words. His own rhythm is long connected sentences joined with because, so and which, explaining as they go.

In a client document the client is you and the firm is we. First person singular is reserved for Debo's own letters. Name the client's people by role and warmly, so that a finding about a system never reads as a finding about a person. We do not perform enthusiasm, we do not use exclamation marks, and we say what we cannot tell them, because candour about the limits of evidence reads as integrity and it protects the firm.

Two hard rules. Never use an em dash, in a document, a headline, a message or interface text; use a comma, a full stop, a colon, or restructure the sentence. And never name our platforms in client body copy.

7.6 Email and sending

The sender of record is hello@consultforafrica.com, and that matters because it is the mailbox Debo checks, so anything sent as something else leaves no evidence where he will look for it. Bulk and outreach sending goes through the transactional mail provider's interface only, because the ordinary office mail system throttles batches and starts refusing them partway through, and a half sent campaign is worse than an unsent one. And before any bulk send, send one real message to a real destination and confirm the record actually arrived.

7.7 Deliverables

Client documents are built as PDFs from scripts in the repository and land in the documents folder, which is how a figure in a summary cannot disagree with the figure in the full document. They are not web pages and they are not pasted into email bodies.

For a proposal or a pitch to a principal, the deliverable is a landscape deck and the prose document is the leave behind. Reports, letters and memoranda stay as prose. For an early stage prospect, the deliverable is the options note: short, if this then expect that, ranges rather than point estimates, no fee table, and explicitly not a bankable case.

Part Eight: What is held until January, and your first ninety days from there

8.1 Why this part is split

You are full time from January, and a good deal of what Part Four and Part Five describe has a date on it before then. So this part separates three things that would otherwise blur: the work that cannot wait and therefore belongs to somebody else for now, the small amount that is genuinely useful for you to do before you start, and the ninety days that begin when you arrive.

The reason to write the first of those down rather than leave it is that an unowned deadline does not announce itself. If a piece of work in this pack has a date in October and no name against it, it will simply pass, and the most likely outcome is that you inherit it in January as a thing that already failed.

8.2 Held by somebody else until January, with the date

Each of these is live now and none of them is yours. **The purpose of this list is for Debo to put a name against every line, and for you to know in January what was done rather than having to find out.**

What	When it bites	Why it cannot wait
The Mezo claim links	**11 October 2026**	The earliest of nine hundred and sixty seven links expire that day, and re seeding afterwards reproduces the same silence. The decision is campaign now or reissue and campaign later, and it needs the engineering side in the room because the seeding cron is deliberately silent.
The Medlyfe Introduces evening	**8 October 2026**	A room full of exactly the Lagos principals the firm wants, with no plan for the guest list afterwards. The follow up is worth more than the evening and it has to be set up before, not after.
Dr Moromoke Ogunko	Already a fortnight old	The only lead the firm holds, qualified, with a stated leader count and timeline, unanswered since 18 September. A reply in January is not a reply.
Pearl Oncology and Osiris Health	Already since May	Both asked how to proceed and neither got an answer. Another three months takes it past the point where a polite reopening is available.
Haven instalment three	**22 October 2026**	A receivable, so Ezinne holds the chase, but it belongs in the same view.
The Aman balance	Past due since 27 September	The reconciliation question comes before the chase. This one touches your own engagement, so it is the likeliest candidate to stay with you.
Duchess and Solace	Open now	Both are waiting on a decision rather than on new work, so a chase costs nothing and silence costs the relationship.

8.3 Worth doing before January, and genuinely not more than this

- Read Parts Four and Five, then Three, then the rest, across the next few weeks
- Write down where you think this pack is wrong, and send it. A view formed before you are inside the machine is worth more than one formed after, and it is the only time you will have it
- Keep the Aman engagement moving, including the reconciliation question in section 3.6, because that is already yours
- Settle your terms with Debo, and what if anything you are picking up between now and the start
- Send a CV so the profile gap in section 2.2 is not being chased in your first week

- Agree with Debo who holds each line in 8.2, which is a twenty minute conversation and the highest value thing in this part

Everything else can wait, and should. The platform access, the rate card and the pipeline surface all arrive with the start date.

8.4 Month one, from January

- Get every name in section 4.2 onto the pipeline surface with a stage, a next action and a date, and re read Part Five's figures off the platform because they will have moved
- Find out what happened to each line in 8.2, and treat anything that lapsed as the first thing to recover
- Decide what happens to the four stale proposals: sent, rewritten, or closed with a reason
- Agree a qualification standard with Debo and write it down, then apply it to everything in the pipeline
- Open the rate card and read the two calibration points on it before you quote anybody
- Get the CadreHealth employer accounts verified and logged in, and find out why none of them ever came back

8.5 Month two

- Put the Pearl and Osiris conversations back on the table with a current price, and treat Osiris's partnership ask as a separate parallel track
- Build the coaching conversion route: the Maarova reports, the onboarded coaches, and the zero matches between them. Start with Haven, Duchess and House of Refuge
- Force the agency rate card decision, then take Grandville forward
- Hold the HealthStack discovery call
- Put the sales agents to work on something, or close the channel
- Start the named reference work with Nimi, because the engagements going well are the ones to ask and the window closes

Own the pipeline rather than rebuild it. By the end of month two the pipeline surface should be the thing the Monday partner meeting runs off, every opportunity on it should have a stage and a next action with a date, and the Thursday sweep should be finding fewer dropped items because fewer are being dropped.

On the products, pick two and go deep rather than touching five. Whatever the Mezo position is by January, and the Maarova coaching conversion, are the two with the clearest evidence behind them.

8.6 Month three

Three things should be true. The pipeline forecasts something, because it has stages, dates and a qualification standard applied consistently. At least one product has a running campaign with a number attached that is going in the right direction. And the firm has more than one cleared named reference, which is the thing that makes the next twelve months of selling easier than the last twelve.

Appendix A: The directory

Confirm titles and reporting lines with Debo when you start, because some of these are practice or faculty roles rather than full time posts, and several will have moved by January.

Person	Role	Contact
Dr Debo Odulana	Founding Partner	debo.odulana@consultforafrica.com
The firm	Sender of record for anything outbound	hello@consultforafrica.com

Person	Role	Contact
Partnerships	Second channel of record	partnerships@consultforafrica.com
Tito Ipinmoye	Director of Product and Strategy	On the platform
Toluwanimi Olayeni, Nimi	Marketing and communications lead	On the platform
Ejiro Alechenu	Marketing	ejiroalechenu0@gmail.com
Oludayo Olugbamigbe	HR Practice Lead	Confirm with Debo
Dr Olufunbi Okeleye	Engagement Manager	engagement.management@consultforafrica.com
Ezinne Orji	Executive Assistant, Office of the Founding Partner	ogeriorji845@gmail.com
Abigail Oladejo	Administrative Assistant	abigail.ayomide04@gmail.com
Kola Momodu	Arabella on site Project Owner	Confirm with Debo
Dr Chito Nwana	Principal, Arabella Women's Health	Missing from our records
Dr Itunu Akinware	Group Chief Executive, Medbury	On the client record
Dr Chinwe Kpaduwa	Aesthetics clinical partner, Medbury venture	On the client record
Zayyad Abdulrahman	Chief Financial Officer, Aman HMO	On the client record
Abubakar Hassan Muhammad, Sadeek Hassan	Chief Executive, Aman HMO	Through Debo
Dr Uju Rapu	Chief Executive, Belfiore Medical	On the client record
Dr Bola Akinola	Osteon Clinics	Missing from our records
Kabir Aregbesola	Haven Paediatric Centre, main contact	kabir@aurorahills.co
Usman Gumi	Haven, copied on invoices	usman.g@aurorahills.co
Favour Olewuenyi	Afya Care, joint governance committee	favour.olewuenyi@afya.care
Dr Oby Clems-Anunwa	Solace Healthcare, Games Village	Confirm with Debo
Mrs Ladipo	Duchess International Hospital	On the client record
Dr Kunle Ajayi	Clearview Fertility and Hospital	Confirm with Debo
Fola Laoye	Iwosan Lagoon, IASO Medipark	Through Debo only
Tosin Runsewe	Afya Care, board chairman	Through Debo only

Person	Role	Contact
Dr Adaorah Enyi	Pearl Oncology, our introducer	dradaorah@pearloncology.com
Mrs Oluwakemi Ayorinde	Pearl Oncology, HR Manager	oluwakemi.ayorinde@pearloncology.com
Ayoola Oladipupo	Chief Executive, Osiris Health	ayoola.oladipupo@osirisng.com
Dr Moromoke Ogunko	Ifako Ijaiye General Hospital, the one open lead	romzylarom85@gmail.com
Dr Francis Korie	CadreHealth's only paying member	frank.korie@gmail.com
Njide Ndili	President, Healthcare Federation of Nigeria	Through Debo only

A note on one address. Debo holds two platform accounts and only the consultforafrica.com one reaches the elevated pages. When he says he cannot see something, check which account he is signed into before you check anything else.

Appendix B: Glossary

Term	What it means here
Audit, organisational	Our structured diagnostic across ten domains of a facility, ending in a report and a set of board decisions
Build and operate	We design and set up a facility or venture, then run it under a management agreement
CAC	Corporate Affairs Commission. Incorporation and company filings
Cadre	One of the sixteen registered healthcare professions in Nigeria. The organising unit of the workforce platform
Contingency, recruitment	A fee payable only on a successful placement, as against a retained search
CSSD	Central sterile services department. Where instruments are sterilised, and a catastrophic risk point in surgery
Day case	A procedure where the patient goes home the same day
DoA	Delegation of authority. The written list of what a board keeps and what it delegates
EBITDA	Earnings before interest, tax, depreciation and amortisation. The operating profit measure in every one of our models
EMR	Electronic medical record
Factoring	Advancing cash against invoices or claims not yet paid. The Mezo growth plan line
Fractional leadership	A senior operator placed part time into a client, such as an interim chief operating officer
Give to get	Unlocking content by contributing content. How the salary map and the hospital reviews work

Term	What it means here
HEFAMAA	The Lagos State facility monitoring and accreditation agency. Abuja has its own route
HEXACO	The six factor personality model behind Maarova's integrity work
HMO	Health maintenance organisation. A health insurer, and the payer behind much of a hospital's revenue
MDCN	Medical and Dental Council of Nigeria. Registers doctors and accredits internship places
Medipark	A campus of several independent clinical businesses sharing a building and its services
MOU	Memorandum of understanding
NDPA 2023	Nigeria Data Protection Act, which replaced the 2019 regulation. Say NDPA, not NDPR
Network adequacy	Whether an insurer's members can actually reach an appropriate provider quickly. The Aman reframe
NICU	Neonatal intensive care unit, and the highest value admission in a paediatric building
Options note	The stage one deliverable: short, ranged, honest about what is unknown, no fee table
Panel, insurance	The list of facilities an insurer will pay. Being on it is the commercial gate
Resourced days	How we price delivery work, as named people for a number of days at published rates, plus a programme rate
Retained search	A search paid partly up front, used for scarce or strategic roles
Secondment	Someone employed by one party and directed by another. The structure of the two Medbury operating officers
Sessional	A doctor who books a session in a clinic rather than being employed by it
SPV	Special purpose vehicle. A company formed for one venture
Stream, Maarova	Recruitment, development or intelligence. Decides which assessment modules run
Takaful	Islamic cooperative insurance. The structure Aman is built on
Tariff	A facility's price list, line by line
Thurstonian IRT	The scoring method behind forced choice assessment. Harder to fake than a rating scale
Ubuntu framework	Maarova's coaching frame, built on collective impact rather than individual achievement
VMI	Vendor managed inventory. The supplier holds the stock and is paid on use, which frees working capital
WHT	Withholding tax. We deduct five per cent from contractor payments and remit it

Appendix C: The channels, on one page

Every route to market the firm has, and the honest state of each.

Channel	What it is	State
Debo's network	Referral from Cedarcrest, Evercare, Doctoora and board seats	The entire current book. Not scalable and not recorded
Pipeline surface	Leads, discovery calls, proposals on the platform	One lead, four stale drafts, zero discovery calls
Maarova demo request	Inbound form, routes to a lead	Works. One request received. Never answered
CadreHealth register	Ten thousand records, one thousand registered	Growing fast. The outreach engine is running
CadreHealth employer side	Search first recruiter product	Four accounts, none verified, none ever logged in
Mezo claim links	Nine hundred and sixty seven open places	Expiring from 11 October. Never campaigned
Agent channel	Commission based field agents with payouts	Four approved agents, one opportunity, zero deals
Partner firm portal	Deploy our bench into other firms' projects	Built. Zero partner firms
Marketing agency	Positioning and acquisition for providers	No rate card, so nothing can be quoted
Published proof	Insights, case studies, named references	Ten case studies written, none published. One reference cleared
Institutional convening	The federation, the Doctors Foundation for Care, policy work	Real standing, no commercial route attached

Appendix D: The standing checklists

Before you write to anybody outside the firm

- Ask Debo how he knows them
- Check the client or lead record for what we have already sent them
- Check whether they hold a platform, portal or product account already
- No platform names in the body copy
- No em dash
- No price unless Debo has cleared the number
- Sender is hello@, and the send is logged in Communications with a next action

Before you quote a price

- Read the figure off the current source, not from a document or from memory
- Confirm the standard, the single named concession and the net

- Confirm what is excluded, particularly tax and government or regulatory fees
- Confirm the payment shape: what is due when, and what triggers each part
- Terms block present: non refundable, free substitution, one reschedule
- Nothing from a private negotiation guide has leaked into the client document

Qualifying an opportunity

- Who actually decides, and have we met them
- What happens if they do nothing, because that is the real competitor
- Is there money, and is it in this budget year
- Can we deliver it, confirmed with Tito or Oludayo rather than assumed
- What is the stage two of this, because the first sale is rarely the business
- Stage, next action and date on the pipeline record, and a qualification note written

Before any bulk send to a product audience

- Suppression list current, and the historical back fill done
- Salutation built with the helper, not with a last name field
- One real message sent to a real destination and the record checked
- Sending through the transactional provider, never the office mail system
- Senior medic register, and the aspirational framing where the audience is consultants
- Debo has seen the copy

Appendix E: Where things live

Client documents are PDFs in the repository's documents folder, organised by client, each generated by a script so a figure cannot drift between a summary and a full document.

The ones most worth reading before January, in this order: the marketing handbook and the marketing operating brief, which cover the four brands and the rules in more depth than Part One does here; the Aman provider strategy, which is your own practice and the best worked example of the reframe; the recruitment pricing memo, remembering that the headline rate moved in September; the Maarova methodology brief and the pricing strategy note; the Medbury division memorandum and its one page summary, for the payment terms Debo now wants everywhere; the Haven growth strategy, as the firm's best turnaround case; and the Mezo handoff note, which is how a doctor actually gets a place.

Client facing project links are single unindexed pages carrying everything a client needs instead of six attachments. Two are live, for Arabella and for Osteon.

Surveys and their responses sit on the platform under the admin surveys reader, and response counts are the thing to check weekly during an audit.

Product state is best read off the platform rather than off any document, because every figure in Part Five will have moved. Maarova organisations, assessment sessions and reports are under the Maarova admin area. CadreHealth professionals, employers, mandates and outreach are under the CadreHealth admin area. The Mezo survey responses are under the Mezo survey reader. The Mezo register itself is in a separate system and needs engineering to query.

Business Development and Growth, Consult for Africa. Internal. Prepared 2 October 2026 for a January start, with every figure read off the live platform on that date, which means Part Five in particular is a snapshot of a moving thing. Revised the same day to separate what is held until January from what begins when you arrive. If anything in this pack turns out to be wrong, say so, and it gets corrected rather than worked around.